ENSIGN SERVICES

POSTette: Progress Report

Reference: Rehabilitation Services P&P- Policy 235 –The Progress Report; Medicare Benefit Manuals Chapters 8 & 15

Progress Report

The progress report provides justification for the medical necessity of treatment. For Medicare payment purposes, information required in progress reports shall be written by a clinician that is, the therapist who provides the services and supervises an assistant.

The minimum progress report period shall be at least once every 10 treatment days, unless more frequently required by state law/practice acts. The day beginning the first reporting period is the first day of the episode of treatment regardless of whether the service provided on that day is an evaluation, re-evaluation, or treatment.

Documenting Clinician Participation in Treatment in the Progress Report. Verification of the clinician's required participation in treatment during the progress report period shall be documented by the clinician's signature on the treatment note and/or on the progress report. (Note: If adding a narrative TENs do not simply state "see progress report". Documentation must include information related to the treatment provided).

Assistant's Participation in the Progress Report. PTAs or OTAs cannot document any element of the progress report. Any PTA or OTA documentation of progress is separate documentation in a treatment encounter note. Only the clinician can complete the progress report during each progress report period.

Content of Clinician Progress Note:

Assessment of improvement, extent of progress (or lack thereof) toward each goal. **Tip: use comments section to add more skilled intervention details such as**:

- Changes in the types or level of assistance required to perform functional tasks.
- Changes in the types of assistive devices
- Improvement in rating of reported pain levels and changes in the ability to perform tasks given the reduction of pain (i.e., Ability to sit for a duration of time because of pain reduction)
- Additional documentation for medically complex, cognitively impaired or patients not showing significant progress at the time the report is completed.
- Any complicating medical factors

Plans for continuing treatment, reference to additional evaluation results, and/or treatment plan revisions such as any changes to goals.

Functional documentation is required as part of the progress report at the end of each progress reporting period. Assure that documentation justifies the necessity of the services provided during the reporting period, particularly when reports are documented at the minimum frequency.

Justification for treatment must include objective evidence or a clinically supportable statement of expectation that:

- In the case of rehabilitative therapy, the patient's condition has the potential to improve or is improving in response to therapy, maximum improvement is yet to be attained; and there is an expectation that the anticipated improvement is attainable in a reasonable and generally predictable period of time.
- In the case of maintenance therapy, treatment by the therapist is necessary to maintain, prevent or slow further deterioration of the patient's functional status and the services cannot be safely carried out by the beneficiary him or herself, a family member, another caregiver or unskilled personnel.

A re-evaluation is not required before every progress report routinely but may be appropriate when assessment suggests changes not anticipated in the original plan of care.

The Plan for Continuing Skilled Intervention:

- List any diagnostic recommendations (e.g., MBSS; BERG; etc.)
- Address any specific education required.
- Provide justification to continue intervention.
- **Address WHY** clinically complex or cognitively impaired patients not demonstrating significant progress will remain on caseload.

Justification Statements are IMPORTANT!

- Why does skilled intervention need to continue?
- What skills were demonstrated/ achieved during the progress note reporting period? (If not clear in the goals summary, additional information needs to be added to support continuation of services.)
- What deficits remain that can benefit from skilled intervention?
- What services will be provided to support continuation of skilled intervention?

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Strong justification statements enhance the clinical picture and are critical to supporting skilled intervention.

- If progress is not significant between reporting periods, change the focus of the justification statement to why the patient is remaining on therapy services.
- Do goals need to be modified or changed? Breaking goals into smaller increments helps show progress that is not identified in a larger more complex goal.
- How does the documentation differentiate need for continued skilled vs. a plateau in progress?
- If any barriers to progress are due to a cognitive issue – documentation must support that the patient can benefit from continued skilled services.

Justification Examples:

"Pt. continues to demonstrate reduced mobility and an altered gait increasing risk for falls. In addition, pt. exhibits low back pain due to poor posturing and compensation with gait and mobility. Pt. is still unable to bear weight and has significant decreased ROM of R knee. Pt. has, however, significantly increased tolerance to a more aggressive intervention and has reduced the amount of pain medications taken PRN."

"The patient has transitioned from honey thick liquids to nectar thick liquids and has increased the number of CCs consumed at meals. The patient is unable to advance to thin liquids due to decreased laryngeal strength and delayed swallow. Continued speech-language pathology services are medically necessary to improve laryngeal strength and function; train compensatory swallow strategies and assess ability to tolerate advanced textures without signs/symptoms of aspiration."

"Patient demonstrates improvement with gait quality, increased LE strength and sensation, improved static and dynamic standing balance and has not had any falls since PT intervention. Continued PT intervention is required to address transition from FWW to single point cane; improved gait patterns on a variety of surfaces in preparation for community ambulation; and patient's ability to independently self-correct improper gait patterns."

"Patient presents with several co-morbidities that impact the length and duration of therapy. Continued skilled intervention will address decreasing patient's RPE while stabilizing O2 for greater independence of functional tasks. Patient requires the skills of a therapist to reach the goals set to maximize independence for discharge."

"Patient has made improvement with LE strength, sensation, gait quality and balance. Patient has not had any falls since initiation of intervention. Goals 1 and 3 have been met. Patient continues to demonstrate inability to progress to a single point cane and demonstrates loss of balance during high level gait activities. Continued physical therapy is medically necessary to improve gait patterns and balance to progress to independent mobility to discharge to prior level of function. New goal of _____ will be added to plan of care."

"Progress is limited by underlying conditions of _____ and _____ requiring modification of the treatment and training of tasks on a daily basis for patient to return to _____level of independence. Continuation of skilled intervention is necessary to _____. Or: *This intervention cannot transfer to nursing due to _____."*

Additional Tips:

- Including objective evidence such as standardized patient assessment instruments, outcome measurements tools or measurable assessments of functional outcome will enhance the justification for needed therapy.
- Modification of short-term goals or treatment plan supports ongoing skilled interventions.
- Documenting the skilled services provided supports need for services.
- Documenting improvements in functional skills or providing additional documentation for patients with limited progress helps with justification of services.
- Documenting current deficits or barriers and including how the barriers will still support the need for skilled intervention will enhance justification.
- Document the justification for continuation even if progress is clearly identified with the goals data.

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